



Public Competition Assessment

19 September 2007

Healthscope Limited - proposed acquisition of Symbion Health Limited's pathology, medical centre and imaging businesses

Introduction

1. On 15 August 2007, the Australian Competition and Consumer Commission (**ACCC**) announced its decision not to oppose the proposed acquisition of Symbion Health Limited's (**Symbion's**) pathology, medical centre and diagnostic imaging businesses by Healthscope Limited (**Healthscope**) (**proposed acquisition**), subject to a section 87B undertaking accepted by the ACCC on 15 August 2007 (**the undertaking**). The ACCC was of the view that the proposed acquisition, in conjunction with the undertaking, would be unlikely to have the effect of substantially lessening competition in the state market for the supply of pathology services in Victoria in contravention of section 50 of the *Trade Practices Act 1974* (**the Act**).
2. The ACCC also formed the view that the proposed acquisition was unlikely to have the effect of substantially lessening competition in any other state market for the supply of pathology services where Healthscope's and Symbion's pathology operations overlap.
3. The ACCC formed its view on the basis of the information provided by the merger parties and information arising from its market inquiries. This Public Competition Assessment outlines the basis on which the ACCC has reached its decision on the proposed acquisition, subject to confidentiality considerations.

Public Competition Assessment

4. To provide an enhanced level of transparency and procedural fairness in its decision making process, the ACCC issues a Public Competition Assessment for all transaction proposals where:
 - a merger is rejected;
 - a merger is subject to enforceable undertakings;
 - the merger parties seek such disclosure; or
 - a merger is approved but raises important issues that the ACCC considers should be made public.

5. This Public Competition Assessment has been issued because Healthscope's proposed acquisition of Symbion's pathology, medical centres and imaging businesses is subject to a court enforceable undertaking and is considered to raise issues that should be made public.
6. By issuing Public Competition Assessments, the ACCC aims to provide the market with a better understanding of the ACCC's analysis of various markets and the associated merger and competition issues. It also alerts the market to the circumstances where the ACCC's assessment of the competition conditions in particular markets is changing, or likely to change, because of developments.
7. Each Public Competition Assessment is specific to the particular transaction under review by the ACCC. While some transaction proposals may involve the same or related markets, it should not be assumed that the analysis and decision outlined in one Public Competition Assessment will be conclusive of the ACCC's view in respect of other transaction proposals, as each matter will be considered on its own merits.
8. Many of the ACCC's decisions will involve consideration of both non-confidential and confidential information provided by the merger parties and market participants. In order to maintain the confidentiality of particular information, Public Competition Assessments do not contain any confidential information or its sources. While the ACCC aims to provide an appropriately detailed explanation of the basis for the ACCC decision, where this is not possible, maintaining confidentiality will be the ACCC's paramount concern, and accordingly a Public Competition Assessment may not definitively explain all issues and the ACCC's analysis of such issues.

The parties

The acquirer: Healthscope Limited (Healthscope)

9. Healthscope is a public company listed on the Australian Stock Exchange. Healthscope is involved in the following Australian businesses:
 - The management of private medical, surgical, rehabilitation and psychiatric hospitals in Australia.
 - The operation of pathology businesses in all Australia states and territories except the Northern Territory. Healthscope's pathology businesses operate under the names General Pathology Laboratories, Cutaneous Pathology and Mount Pathology (Western Australia), Gribbles Pathology (Victoria, Queensland and South Australia), and Davies Campbell de Lambert Pathology (New South Wales and Australian Capital Territory).
 - The operation of specialist skin cancer clinics across Australia.
 - A minority, non-controlling investment in NM&IG Medical Pty Ltd. NM&IG Medical Pty Ltd operates a number of medical clinics.

The target: Symbion Health Limited (Symbion)

10. Symbion is a public company listed on the Australian Stock Exchange. Symbion's business units include Australian pathology businesses, medical centres, diagnostic imaging, pharmacy and consumer products.
11. Symbion's pathology business operates under the names Dorevitch Pathology (Victoria, New South Wales), Gippsland Pathology Services (Victoria), Lavery Pathology (New South Wales, Australian Capital Territory), QML Pathology (Queensland and Northern New South Wales) and Western Diagnostic Pathology (Western Australian and Northern Territory).

Other industry participants

Sonic Healthcare Limited (Sonic)

12. Sonic is a publicly listed pathology service provider operating in all states and territories of Australia. Sonic is the largest pathology service provider in Australia.

Analytical Laboratories Pty Ltd (ARL)

13. ARL is a privately owned pathology service provider based in Victoria. ARL supplies general pathology services to general practitioner and specialist referred patients.

St John of God Health Care (SJGHC)

14. SJGHC is a not-for-profit organisation which provides pathology services in Western Australia and Victoria. SJGHC supplies general pathology services to general practitioner and specialist referred patients and supplies pathology services to private hospitals (typically owned by SJGHC). SJGHC also supplies pathology services to public hospitals under contract in Victoria.

Primary Health Care Limited (Primary)

15. Primary is a publicly listed medical centre operator and pathology service provider based in New South Wales with limited participation in Victoria and Queensland. Primary supplies general pathology services to general practitioner and specialist referred patients.

The proposed transaction

16. Healthscope proposed to acquire all of Symbion by way of a scheme of arrangement (the **Scheme**). Healthscope had entered into an agreement with Ironbridge Capital and Archer Capital which contemplated the subsequent sale of Symbion's pharmacy and consumer businesses to Ironbridge Capital and Archer Capital once the Scheme had been implemented. Healthscope proposed to retain Symbion's pathology, medical centre and diagnostic imaging businesses.

17. Symbion did not receive adequate votes at its shareholder meeting of 11 September 2007 in favour of the Scheme. As a result, the Scheme could not be implemented.

Areas of overlap

18. The main area of overlap between Healthscope's and Symbion's businesses is in the provision of pathology services.
19. The ACCC notes that there is no overlap between Healthscope's business and Symbion's imaging business. Symbion's imaging business is not discussed further in this Public Competition Assessment.
20. The ACCC also notes that Healthscope does not have a controlling interest in any medical centre business in Australia. Given that there is no overlap between Healthscope's business and Symbion's medical centre business, Symbion's medical centre business is not discussed further in this Public Competition Assessment.

Timing

21. The following table outlines the timeline of key events in this matter.

Date	Event
01-May-2007	ACCC commenced review under the Merger Review Process Guidelines.
18-May-2007	Closing date for submissions from interested parties.
12-Jun-2007	Former proposed date for announcement of ACCC's findings, amended to allow for further market inquiries.
20-Jun-2007	ACCC published a Statement of Issues outlining preliminary competition concerns. New round of market inquiries commenced.
11-Jul-2007	Closing date for submissions relating to Statement of Issues.
12-Jul-2007	ACCC requested further information from Healthscope, response expected by 20 July 2007. ACCC expects to announce findings within three weeks of Healthscope's response. ACCC timeline suspended.
23-Jul-2007	ACCC received further information from Healthscope. ACCC timeline recommenced.
26-Jul-2007	Draft 87B undertaking proffered by Healthscope. ACCC commenced market inquiries.
03-Aug-2007	Closing date for submissions relating to draft 87B undertaking.
15-Aug-2007	ACCC announced it would not oppose the proposed acquisition, subject to accepting court enforceable undertakings.

Industry background

Pathology services

22. Pathology is the branch of clinical medicine concerned with understanding the causes and processes of diseases and providing scientific analysis which forms an essential basis for medical diagnosis and treatment. Pathology testing involves the analysis of medical samples, for example blood cell counts, blood chemistry analyses and microbiology cultures, which are collected from patients. Pathology testing is required by patients and doctors in both community and hospital settings and is provided by both private and public pathologists.
23. Pathology can be divided into recognised sub-specialties, namely chemical pathology, haematology, microbiology, histopathology, immunology and cytopathology. Different sub-specialties are automated to varying degrees with more routine testing in chemical pathology, haematology and immunology being highly automated and more specialised testing in histopathology requiring greater pathologist supervision and analysis of test results.

Pathology providers

24. Apart from Healthscope and Symbion, there are other private pathology service providers in Australia including Sonic, ARL and SJGHC listed in the section *Other Industry Participants* above. The ACCC also notes that there are public pathology providers, who may undertake some private pathology work, for example, services to private patients in public hospitals. Public pathology providers, which are publicly funded, include providers such as Southern Cross Pathology, Network Pathology, Eastern Health, Royal Children's Hospital, and PathWest.

Customers

25. The ACCC understands that there are three primary classes of customers for private pathology providers: (i) patients whose tests are ordered by a General Practitioner; (ii) patients whose tests are ordered by a specialist; and (iii) tests for private in-patients in private or public hospitals. It is understood that the majority of pathology tests (around 70 per cent) are ordered by general practitioners, followed by 20 per cent of tests ordered by specialists and the remainder for private in-patients at public hospitals and private hospitals. Each class of customers has particular service requirements, with pathology tests ordered by general practitioners typically involving routine tests whereas pathology tests ordered by specialists can involve more complex testing.
26. The ACCC notes that apart from customers referred by medical practitioners discussed in paragraph 25 above, private pathology providers compete in the provision of pathology tests to commercial customers. Commercial customers include pharmaceutical companies requiring clinical trial testing, corporate customers such as mining companies requiring blood alcohol testing of their staff on-site and commercial insurance companies. The ACCC understands that commercial customers typically tender their pathology test requirements.

Overview of the regulatory framework

27. The pathology industry is characterised by a high degree of regulation. The regulatory framework covers all aspects of pathology service provision. There are interrelationships between the various regulatory arrangements, which impose conditions at every functional level of the industry. Regulatory conditions are imposed on Approved Pathology Authorities (**APAs**), Approved Pathology Laboratories (**APLs**), Approved Pathology Practitioners (**APPs**) and Approved Collection Centres (**ACCs**).

Approved Pathology Practitioners (APPs) and Approved Pathology Authorities (APAs)

28. For a pathology test to be eligible for reimbursement under Medicare, it must be performed by an APP at an APL owned by an APA. The approval processes for APPs and APAs are similar, with both required to submit an undertaking to the Minister of Health and Aging (**Ministerial undertaking**).
29. A company or person can provide a Ministerial undertaking to be registered as an APA. Under its Ministerial undertaking, an APA is responsible for informing Medicare of its financial affairs and complying with all relevant regulation pertaining to the operation of all APLs it operates.
30. An APP must be a medical practitioner with formal qualifications (for example, a fellow of the Royal College of Pathologists) and experience. Under their Ministerial undertaking, an APP is responsible for assuring the quality of pathology tests provided under their supervision.

Approved Pathology Laboratories (APLs)

31. Medicare-eligible pathology tests must be performed in a laboratory accredited under the *Health Insurance (Accredited Pathology Laboratories) Principles 2002* (**'the APL Principles'**).
32. There are five laboratory categories specified in section 17 of the APL Principles. The National Pathology Accreditation Advisory Council (**NPAAC**) establishes the required minimum laboratory standards for Medicare-eligible pathology services rendered in each of the five laboratory categories. The ACCC understands that the most common APLs are either categorised 'G' APLs, encompassing categories 'GX' and 'GY', or categorised as 'B' APLs. Healthscope and Symbion predominantly operate category 'G' and category 'B' APLs, which have been the focus of the ACCC's analysis of the proposed acquisition.
33. The NPAAC revised the requirements for accreditation of pathology laboratories in 2006, with the changes coming into effect on 1 August 2007 (**'the NPAAC APL requirements'**).
34. The following table sets out the requirements for the operation of both category 'G' and category 'B' APLs and the interrelationship between them under the NPAAC APL requirements:

APL Category	Requirements
Category 'G' (General) encompassing: Category 'GX' Category 'GY'	A category 'G' laboratory is an unrestricted category where the full suite of pathology tests can be performed. A category 'GX' laboratory has more than two full-time equivalent (FTE) pathologists. A category 'GY' laboratory has at least one, and not more than two, FTE pathologists.
Category 'B' (Branch)	A category 'B' laboratory is an unrestricted category where the full suite of pathology tests can be performed. A category 'B' laboratory must have a documented agreement with a category 'G' laboratory to ensure that the range of pathology tests provided and the standard of work in the laboratory is under the direction and control of a designated supervising pathologist or senior scientist of a category 'G' laboratory. A category 'B' laboratory is either: - an integral part of a category 'G' laboratory, apart from its geographic location; or - a part of a regional pathology service.

35. The ACCC notes that the substantive changes to the NPAAC APL requirements relate to the requirement for a minimum of one full time equivalent (**FTE**) pathologist in order to operate a category 'GY' laboratory. Under the former requirements, a category 'GY' laboratory could operate under the FTE supervision of a senior scientist with limited input from a pathologist.
36. The National Association of Testing Authorities (**NATA**) is the entity responsible for conducting the inspection of laboratories and the administration of the NPAAC requirements for the accreditation of APLs.
37. The NPAAC APL requirements specify the supervisory role of a category 'G' APL in the management and operation of a category 'B' APL.

38. APAs typically operate a hub and spoke model of APLs, with a *hub* category 'G' APL connected to *spoke* category 'B' APLs located throughout the region in which the APA has a presence through ACCs and/ or doctor collection services.

The new licensing regime for approved collection centres

39. The collection of pathology specimens can be done at ACCs, at the referring doctor's practice, the patient's home or nursing home. The collection of pathology samples can be done by the patient themselves, a phlebotomist, the referring medical practitioner, a practice nurse or a pathologist.
40. The most common medical sample collected for pathology testing is a blood sample. Samples collected at ACCs typically are blood samples. Niche pathology providers who supply certain sub-specialties, for example histopathology, do not require blood samples and instead rely on doctor collects for medical tissue samples and do not typically require ACCs for the collection of their pathology samples. Conversely, general pathology providers who supply the more common sub-specialties, for example haematology, do require blood samples and rely on ACCs for the majority of their community pathology samples.
41. An APA must obtain approval for the operation of an ACC if the APA wants to bulk bill any of its patients who attend the ACC. Such approval is granted by the Minister for Health and Aging under the *Health Insurance Act 1973*. Gaining such an approval is analogous to obtaining a licence to operate a facility as an ACC. Without obtaining such approval, an APA cannot operate a licensed ACC. Pathology samples collected at an unlicensed collection centre are not eligible for bulk billing payments..
42. The NPAAC has published guidelines for ACCs which establish the minimum requirements to apply to collection facilities utilised in the collection of pathology samples for Medicare-billed testing in APLs (**'the NPAAC ACC Guidelines'**). The NPAAC ACC Guidelines contain conventions regarding the premises, equipment and staffing of a collection facility operating under an ACC licence.
43. Up to 1 July 2007, ACC licences were allocated under the *Health Insurance (Eligible Collection Centres) Approval Principles 2005* (**former ECC principles**).
44. Under the former ECC principles, ACC licences were allocated according to the number of Patient Episodes an APA received in a preceding financial year, with the allocation made on 1 July of every year.
45. Under the former ECC Principles, a new entrant APA with a category 'G' APL received a minimum of two ACC licences for the first year of operation.
46. As of 1 July 2007, the *Health insurance (Eligible Collection Centres) Approval Principles 2007* (**current ECC principles**) have been in place.

47. The current ECC Principles have changed the regulatory environment in four significant ways relevant to the competition analysis of the proposed acquisition. These changes are set out below:
- a. Change One – A new entrant with a category 'G' APL will receive a total of *four licences plus an additional licence to operate an adjoining ACC at its APL premises* in its first year of operation. This is *an increase of two* for a new entrant when compared to the previous minimum floor of two licences;
 - b. Change Two – ACC allocations will no longer be calculated by reference to the number of patient episodes an APA receives in the preceding financial year. Instead, an APA with a category 'G' APL will have its yearly nominal allocation of licences calculated *on the basis of its previous year allocation*. An additional allocation will be calculated; proportional to the national population growth rate, with APAs guaranteed a minimum of one additional allocation.
 - c. Change Three – Under the former ECC Principles, an existing APA received an *additional two ACC licence allocations for every additional category 'G' APL* it established. Under the current ECC Principles, an existing APA will receive *no ACC licences* for establishing an *additional Category 'G' APL*. Where an existing APA has a shareholding of more than 20 per cent in a new entrant APA, the new entrant APA is deemed related to the existing APA and will receive no ACC licence allocations for any category 'G' APL it establishes, including its first category 'G' APL.
 - d. Change four – Under the former ECC Principles, trading in ACC licences was permissible between APAs. Under the current ECC Principles, such trading is prohibited.

Market inquiries

48. The ACCC conducted extensive market inquiries with a range of industry participants, including competitors, potential competitors, customers, input suppliers, industry bodies, other regulatory agencies and other interested parties. Submissions were sought in relation to the substantive competition issues and the undertaking.

Statement of Issues

49. The ACCC published its Statement of Issues on 20 June 2007 identifying a number of competition issues.

Issues unlikely to raise concerns

50. The ACCC noted the following issues as not being of concern:
- a. The sale of Symbion's medical centres to Healthscope;

- b. The sale of Symbion's diagnostic imaging business to Healthscope;
and
 - c. Vertical integration of Symbion's pathology and diagnostic imaging businesses with Healthscope's, 48 private hospitals in Australia
51. The ACCC's market inquiries pursuant to the Statement of Issues confirmed the ACCC's findings set out in paragraph 50 above.
52. In the Statement of Issues, the ACCC delineated state based markets for the provision of general pathology services and a national market for the provision of pathology services to public hospitals.
53. Market inquiries did indicate some uncertainty concerning the geographic dimensions of the public hospital pathology services market.
54. Market inquiries indicated that public hospital pathology markets may be delineated on a state by state basis. Reasons supporting a more narrow delineation included the differing requirements of each state Health Department in the tendering process for the provision of pathology services in its public hospitals and the importance of brand reputation and presence on a state-level when a pathology provider bids for a public hospital pathology services contract.
55. However, given that no competition issues arose in the market/s for public hospital pathology services, the ACCC did not consider it necessary to come to a definitive view on the geographic dimension of this market/s.

Issues that may raise concerns

56. The Statement of Issues noted the following issue that may raise concern:
- a. The merged entity's increased market share in the Victorian veterinary pathology services market.
57. The ACCC sought further information on the availability of substitute veterinary pathology service providers to the merged entity in Victoria and the barriers to entry into the market.
58. Market participants did not raise concerns with respect to the Victorian Veterinary Pathology Services market during the Statement of Issues phase of market inquiries.
59. In any event, under the undertaking (discussed in detail below), Healthscope proposes to divest some Symbion pathology businesses in Victoria as part of other divestitures, which will include the divestiture of veterinary assets. This resolved any competition concerns in the Victorian veterinary pathology market.

Issues of concern

60. The Statement of Issues identified the following issues of concern:

- a. The proposed acquisition is likely to raise competition concerns in the Victorian pathology services market given that the merged entity will be the largest pathology services provider in a market that is characterised by high barriers to entry and expansion and few substitute pathology service providers. The ACCC considers that smaller, niche pathology service providers are unlikely to provide a competitive constraint on the merged entity. **(Unilateral effects in the Victorian market for the provision of community pathology services)**
 - b. Post acquisition market structures in Victoria, Queensland and Western Australia are likely to increase the ability and incentive for coordinated conduct between the merged entity and Sonic. **(Coordinated effects in the Queensland, Victorian and Western Australian markets for the provision of community pathology services between the merged entity and Sonic)**
61. The ACCC sought further information and evidence from market participants to resolve these issues.

Unilateral effects in the Victorian market for the provision of community pathology services

Market definition

62. In defining relevant markets, the ACCC adopts a purposive approach. In the Statement of Issues, the ACCC delineated state based markets for the provision of pathology services to private hospital in-patients and patients referred by general practitioners and specialists. In its consideration, the ACCC noted the following:
- a. Whilst some pathology samples may be transported interstate for testing, in general, the proportion of samples tested interstate appears to be relatively low.
 - b. The urgent nature of some tests (eg blood matching) requires prompt testing and reporting of results.
 - c. A state based presence may be necessary to facilitate awareness with doctors and develop relationships.
 - d. Brand development and retention of state branding by private pathology providers with national operations.
63. The ACCC's inquiries confirmed that the provision of pathology services to private hospital in-patients and patients referred by general practitioners and specialists **(community pathology services market)**¹ is a separate product market to the provision of pathology services to public hospitals.

¹ General pathology services markets can also be described as community pathology services markets.

64. The ACCC's inquiries revealed that the provision of community pathology services can be split into two interrelated functional levels – namely, the collection of pathology samples and the testing of pathology samples.
65. For GP referred tests the collection of pathology samples tends to take place within a narrow radius of the patient's residence. Market inquiries revealed that the catchment area is narrower in metropolitan areas when compared to rural and regional areas. The ACCC understands that in regional centres, the catchment area is wider given the dispersed nature of rural populations
66. The ACCC considered whether the markets for the provision of community pathology services should be delineated as state or regional geographic markets. In the circumstances of an integrated market operating at both regional and state levels, the ACCC considered on balance that state based market definitions were appropriate in this matter. The ACCC noted that in some circumstances, the integrated market for the provision of community pathology services may be delineated more narrowly than a state based market.
67. The regulatory framework associated with the granting of ACC licences inextricably links the operation of an APL with the operation of ACCs. The ACCC notes that there is support for delineating the collection of pathology samples. However given that the collection and testing of pathology samples cannot be functionally separated, for the purposes of competition analysis state wide markets for the provision of pathology services were adopted.
68. The ACCC considered that a localised collection presence is critical to an APA's ability to compete for patients samples in a region. This localised competition in the collection of patients' samples supports the identification of market segments within the broader Victorian market for community pathology services. These market segments devolve around regional catchment areas, for example the Gippsland segment, the Northern Corridor of Victoria segment (encompassing the region between Bright and Albury/ Wodonga), the Mornington Peninsula segment and multiple segments within metropolitan Melbourne.

Market concentration

69. The ACCC understood that up to 1 July 2007, ACC allocations were made based on the total number of patient episodes collected by an APA (including doctor collects picked up by the APA from medical centres). An APA was allocated an ACC licence for every 14,200 Patient Episode Initiations (**PEIs**) it collected in the year prior to the allocation date (1 July of every year). For example, an APA collecting 71,000 PEIs in a year ending 30 June 2005 would receive five ACC licences for use in the year starting 1 July 2006.
70. Whilst commercial work and other non-Medicare funded pathology tests do not count towards PEI numbers, information at hand to the ACCC indicated that these types of tests account for a comparatively small proportion of tests pathology providers undertake.

71. The ACCC considered that given the total number of ACCs allocated to each APA is a historical reflection on the total PEIs collected by an APA, market share data based on the numbers of ACCs is a relatively accurate measure.
72. The extrapolation of market share data from ACC numbers is suitable for the purposes of analysis. In the future, ACC market share data may not be an accurate measure of concentration. This is because the current ECC Principles will cause the allocation of ACC licences to APAs to be based on pre-existing ACC licences rather than the APA's growth in its actual volume in PEI numbers.
73. According to market share data based on the numbers of ACCs each private pathology provider operates in Victoria, Symbion has a 33 per cent share, Healthscope has a 21 per cent share and Sonic has a 20 per cent share of the Victorian community pathology services market with the remaining 26 per cent shared by smaller pathology providers.
74. The ACCC's inquiries revealed that whilst there are a number of APAs operating in metropolitan Melbourne and Western Victoria, including general and niche private pathology providers, only Healthscope and Symbion have significant operations in the Northern Corridor of Victoria. Similarly, Symbion has a strong presence in the Gippsland region with Healthscope vigorously competing and making inroads into that area in recent times.

Competition analysis

75. Market inquiries revealed that general pathology providers largely compete on service metrics, including the reputation and skills of the provider's employed pathologists, the reputation of the provider, the proximity of the provider's ACCs, the proximity of labs staffed by pathologists, the provider's turn-around times for pathology test results, IT interfacing between the referring practitioner's software and the provider's software, medical litigation risk management and the relationship between the doctor and the provider.
76. The ACCC noted that the above factors vary in importance to different medical practitioners. For example, a skin cancer specialist may view the reputation and skill sub-set of the pathologist as critical when making his/her referral. Whereas, a referring general practitioner may view the proximity of the general pathology provider's ACCs to his/ her patients as an influential consideration.
77. The ACCC considered that whilst individual practitioners' choices are permutations of the factors above, market inquiries confirmed that general pathology providers compete vigorously for referring practitioners' custom through such service metrics.
78. Market inquiries raised the concern that the merged entity may be able to decrease the quality of its services in areas of Victoria where the merged entity would be the sole or dominant general pathology provider.
79. The ACCC noted that since the introduction of Medicare over two decades ago, bulk billing rates for pathology services have been rising in all Australian states and territories. For example, in Tasmania bulk billing rates were lower than

40 per cent in 1984 and have since risen by a significant percentage to the current bulk billing rates of approximately 76 per cent in that state.

80. Bulk billing rates for pathology services are high across all states and territories. The following table shows the level of bulkbilling for pathology services in each state and territory:

<u>State</u>	<u>NSW</u>	<u>VIC</u>	<u>QLD</u>	<u>SA</u>	<u>WA</u>	<u>TAS</u>	<u>ACT</u>	<u>NT</u>
<u>Bulkbilling rate</u>	<u>87%</u>	<u>84%</u>	<u>85%</u>	<u>88%</u>	<u>86%</u>	<u>76%</u>	<u>88%</u>	<u>96%</u>

(Source: Medicare Annual Report 2005-2006)

81. The ACCC did not observe any significant differences in bulk billing rates across states based on the number of APAs servicing that state. For example, the Northern Territory, which has historically been serviced by only one private APA has bulk billing rates which are close to 100 per cent.
82. The ACCC also noted that in the highly concentrated Queensland market for pathology services, the bulk billing rate is comparable to less concentrated markets for pathology services.
83. Market inquiries indicated that there are various factors influencing a pathology provider's decision to bulk bill its patients. Such factors include pressure by referring practitioners, historical bulk billing practices of APAs in that region, the costs of abandoning bulk billing and instituting private billing and the Memorandum of Understanding between the Commonwealth Government, the Australian Association of Pathology Practices, the Royal College of Pathologists Australasia and the National Coalition of Public Pathology, which contains disincentives to all pathology providers to stop bulk billing.
84. The ACCC understood that Healthscope, in Victoria, has a policy of bulk billing and positions itself in the market as a bulk billing APA. The choice of whether a patient is bulk billed for pathology services is at the election of the referring practitioner.
85. Market inquiries also indicated some concern that the merged entity may be able to increase prices or reduce bulk billing rates for the provision of pathology services in areas of Victoria where it would be the sole or dominant general pathology provider.
86. Given the information at hand to the ACCC, the ACCC did not consider it had sufficient evidence to conclude that bulk billing rates would decrease in Victoria or regions of Victoria as a result of the acquisition.

Barriers to entry and expansion

87. In the Statement of Issues, the ACCC noted that barriers to entry and expansion are high in the pathology services industry. Given the contradictory submissions

on barriers to entry and expansion, the ACCC sought further information and evidence from participants and the merger parties.

Costs of establishment

88. Market inquiries indicated that capital costs are not high in establishing an APL which has the capability to test across most or all specialties. Access to equipment is not restricted and new entrants are able to negotiate leases for equipment with reagent suppliers or purchase the equipment outright. Furthermore, it appears that an APA can add to its capacity by purchasing additional components to complement its existing equipment and incrementally increase capacity.
89. The ACCC noted that working capital costs can be significant, particularly with respect to start-up APAs providing general pathology services.

Importance of collection centres

90. ACC licences are critical to a general pathology business, i.e. an APA that is testing across most or all pathology specialties. Sub-specialties such as chemical pathology, microbiology and haematology comprise the bulk of pathology tests (approximately 75 per cent). These sub-specialty tests utilise blood samples, which are typically collected at ACC locations. In order for an APA to effectively compete in the collection of blood samples, the APA must possess ACC licences and site them in suitable locations. ACCs may also serve a branding purpose promoting the APA's services within the locality in which it is located.
91. Certain sub-specialties, such as histopathology, utilise tissue samples which are typically collected by the referring medical practitioner. Niche pathology businesses supplying histopathology services are unlikely to benefit as much as general pathology businesses from siting their ACC licences. However, given that such niche pathology businesses do not constrain general pathology businesses in the provision of the more generic pathology tests, such as chemical and haematology, the ACCC did not consider that a niche pathology business's requirements could be extrapolated as being the requirements facing a general pathology business in the competition for pathology samples.
92. Market inquiries indicated that in regional Victoria, a general pathology provider may be able to successfully commence operations in a given region through winning a pathology services contract with the local public hospital. The general pathology provider may then place ACCs in suitable sites in the region to service the community pathology service requirements of the region.

Requisite scale

93. Market inquiries revealed that economies of scale represent a considerable barrier to entry and expansion for general pathology businesses. Whilst there is some evidence of entry by general pathology businesses, such businesses are typically developed from niche histopathology businesses and face considerable difficulties in growing to a sufficient scale to break-even.

94. As noted above, in order to be eligible for ACC licences, a new entrant must establish a G category laboratory. A G category laboratory must be staffed by a full-time pathologist. Evidence suggests that in order for a G laboratory to be profitable it must achieve significant scale. The ACCC notes that the major pathology providers in Australia use their G category laboratories as the hub in a hub-and-spoke model to take advantage of these economies of scale.
95. Market participants stated that there are significant scale economies associated with viably operating a general pathology business. Market inquiries indicated that a new entrant with five ACCs may find it difficult to obtain a sufficient volume of pathology samples to run a profitable general pathology business.
96. Market inquiries also revealed that a new entrant or existing general pathology business will face difficulties in organically growing their operations to a sufficient scale to pose a competitive constraint on the merged entity.

Merged entity's vertical links acting as barriers to entry and expansion

97. Information at hand to the ACCC confirmed that the merged entity's vertical links in the ownership of private hospitals and medical centres are unlikely to raise competition concerns in the Victorian market for the provision of community pathology services.
98. The merged entity's ownership of medical centres is limited in Victoria. Furthermore, evidence provided to the ACCC indicated that general pathology businesses can and do compete for pathology referrals from all medical centres, regardless of whether a medical centre is owned by a competing pathology business.
99. With respect to the merged entity's ownership of nine private hospitals in Victoria, the ACCC considered that pathology samples from Healthscope's hospitals do not represent a substantial volume of pathology samples generated in the Victorian pathology services market.
100. Given the ready availability of pathology samples from sources other than the merged entity's medical centres and private hospitals in Victoria, the ACCC did not consider that the merged entity has the requisite control of the supply of pathology sample referrals in order to be able to foreclose on competing general pathology providers in the Victorian market for the provision of pathology services.

Shortage of pathologists

101. Market inquiries revealed that there is a shortage of pathologists in Australia. Market inquiries indicate that to an extent, senior scientists are able to take over the supervisory role of pathologists in the operation of some APLs.
102. The NPAAC APL Requirements now mandate the employment of a FTE Pathologist in order for an APA to operate a category 'G' APL. The ACCC noted that senior scientists would not be able to fill a pathologist's role in the operation of a category 'G' APL.

103. The upward pressures on pathologist's wages could translate into a greater requirement for scale in order for a pathology business to recoup the costs of employing a pathologist.

Substitutes

Niche providers

104. Smaller, niche providers provide a limited constraint to larger, general pathology businesses in so far as their service provision overlaps with that of the larger, general pathology businesses. For example, a histopathology business located in Melbourne is likely to provide some constraint to the merged entity in the provision of histopathology services in Victoria. However, the ACCC considered that such a niche provider would not constrain general pathology businesses in the other sub-specialties, for example chemical and haematology.
105. Smaller general pathology businesses may constrain the merged entity to the extent of their collection catchment and through their ability to achieve sufficient scale to operate as a viable ongoing business.
106. The ACCC also noted the existence of general pathology providers such as Sonic, SJGHC and ARL in Victoria posing some constraint to the merged entity in so far as the locations of their collection operations overlapped.

Public providers ability to compete with private providers

107. Market participants expressed the view that public providers' primary focus is the provision of pathology services to public hospital in-patients, with public providers neither operationally capable of servicing both private out-patient and public in-patient requirements nor possessing requisite funding to compete effectively with private providers.
108. Since May 2007 public providers receive a nominal flat PEI fee of \$2.40 regardless of how the sample is collected. This can be contrasted with the PEI fees that private providers receive which range between \$8.40 and \$17.40. A private provider receives \$17.40 PEI fees for every patient's sample/s collected at an ACC. Prior to May 2007, public providers did not receive any PEI fee. The ACCC understood that public providers continue to face a cost disadvantage in supplying pathology services to out-patients when compared to private pathology providers.
109. Therefore, the ACCC considered that public providers pose a limited constraint on private providers in the Victorian market for the provision of pathology services.

Impact of Undertaking

110. In light of the above, the ACCC considered that the proposed acquisition is likely to raise significant competition concerns in the Victorian market for the provision of community pathology services.

111. To address the ACCC's competition concerns in the Victorian market for the provision of community pathology services, Healthscope offered, and the ACCC accepted, undertakings pursuant to section 87B of the Act. Healthscope's undertaking requires it to divest a number of general pathology businesses, namely the Gippsland Business, Benalla Business and Albury Business operated by Symbion and the Wangaratta business operated by Healthscope post acquisition. The divestiture business's operations are in the regions of Victoria identified as the regions where the merged entity would have been the sole or dominant private general pathology provider post-acquisition.
112. A public version of the undertaking provided by Healthscope is reproduced at Attachment A to this Public Competition Assessment. Schedule Two of the undertaking identifies the divestiture business including the number and location of APLs and ACCs operated as part of the divestiture business.

Coordinated effects in the Queensland, Victorian and Western Australian markets for the provision of community pathology services between the merged entity and Sonic

113. In the Statement of Issues, the ACCC sought submissions from market participants on the increased likelihood of coordinated conduct between the merged entity and Sonic in the Queensland, Victorian and Western Australian markets for the provision of community pathology services. The acquisition will result in the same two companies, Sonic and Healthscope, being the major pathology providers in most states. The ACCC was concerned that this may reduce the incentives for aggressive competition by the companies in one state, knowing that their major competitor could retaliate in another state.

Competition analysis

114. In the Queensland market for the provision of community pathology services, despite the high level of concentration, there is competition between general pathology providers for referring practitioners' referrals.
115. General pathology providers also engage in some service differentiation, with different providers establishing reputations for different reasons. For example, a general pathology provider may actively market itself as providing rapid turn-around times on test results whereas another general pathology provider might market itself as employing highly skilled pathologists.
116. Market inquiries yielded few responses on the issue of coordinated conduct between the merged entity and Sonic post-acquisition. Whilst there was some anecdotal concern that the merged entity and Sonic would have an increased incentive to engage in coordinated conduct across the relevant markets post-acquisition, these concerns were not substantiated by supporting evidence.

Conclusion

117. On the basis of the above, including taking into account the undertaking, the ACCC formed the view that the proposed acquisition of Symbion Health Limited's pathology, medical centre and diagnostic imaging businesses by

Healthscope Limited would not be likely to result in a substantial lessening of competition in the Victorian market for the provision of community pathology services in contravention of section 50 of the Act.

118. The ACCC also formed the view that the proposed acquisition would not be likely to result in a substantial lessening of competition in other markets for the provision of community pathology services where the operations of Symbion and Healthscope overlap (e.g. Queensland) in contravention of section 50 of the Act.

**Undertaking to the
Australian Competition and
Consumer Commission**

**Given under section 87B by
Healthscope Limited**

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1. Background

- 1.1 Healthscope and Symbion each operate pathology businesses in markets for the provision of community pathology services and a market or markets for the provision of pathology services to public hospitals in Australia. In the Commission's view, there are state markets for the provision of community pathology services with geographical segments within those markets. The Commission considers it unnecessary to form a concluded view in relation to whether there is a national market for the provision of pathology services to public hospitals in Australia or whether there are state markets for those services.
- 1.2 Healthscope proposes to acquire Symbion's pathology, medical centre and imaging businesses in Australia (the **Proposed Acquisition**). Relevantly, Symbion's pathology businesses compete with Healthscope in markets for the provision of community pathology services.
- 1.3 The Proposed Acquisition is to be implemented by way of a scheme of arrangement under which all of the shares in Symbion will be acquired by Healthscope Investments, a wholly owned subsidiary of Healthscope (the **Scheme**). The Scheme was announced by Healthscope and Symbion to the Australian Stock Exchange on 29 May 2007.
- 1.4 Immediately after implementation of the Scheme, agreements will be entered into between Symbion and companies ultimately jointly owned by funds managed and advised by Ironbridge Capital and Archer Capital for the sale of Symbion's consumer products group and Symbion's pharmacy services and distribution group. The only businesses previously operated by Symbion that will remain in the control of Healthscope are Symbion's pathology, medical centre and imaging businesses.
- 1.5 The Proposed Acquisition raises competition concerns for the Commission in markets for the provision of community pathology services. The Commission is concerned that a participant's ability to compete in markets for the provision of community pathology services in regional areas may be affected by its ability to compete to provide pathology services to public hospitals in the same regional area.
- 1.6 Specifically, concerns have arisen in relation to the Victorian market for community pathology services and in particular, regions within the Victorian market; namely north-eastern Victoria and Gippsland. The Commission is concerned that following the Proposed Acquisition Healthscope will be the sole or dominant provider of community pathology services in those regions and that competitors will have little or no opportunity to compete for the provision of pathology services to public hospitals in the relevant regions making entry or expansion in the market for the provision of community pathology services difficult.
- 1.7 The Commission conducted market inquiries in relation to its concerns. Following those market inquiries, the Commission was satisfied that the Proposed Acquisition would make it unlikely that a competitor to Healthscope could either enter or expand into the relevant market with sufficient scale to constrain Healthscope. Consequently the Commission is

concerned that the Proposed Acquisition is likely to have the effect of substantially lessening competition in the relevant market.

- 1.8 Healthscope does not agree that the Proposed Acquisition will be likely to substantially lessen competition, but has offered this Undertaking in order to address the Commission's concerns. The Undertaking provides for the divestment by Healthscope of sufficient assets to enable a purchaser of those assets to compete effectively in the relevant market.
- 1.9 Pursuant to this Undertaking, Healthscope has agreed to sell the Divestiture Business, comprising the Gippsland Business, Benalla Business and Albury Business operated by Symbion and the Wangaratta Business operated by Healthscope. Healthscope will use its best endeavours to enter into an arrangement with a purchaser for the Divestiture Business prior to the Scheme's implementation date, with the sale to occur the day after the implementation of the Scheme to a purchaser approved of by the Commission. If this does not occur, then Healthscope has agreed to have the Divestiture Business independently managed until it is sold in accordance with this Undertaking. In addition, Healthscope has agreed to sell the Divestiture Business within a confidential timeframe to a purchaser approved of by the Commission. However, in the event that Healthscope does not sell the Divestiture Business within the specified timeframe, Healthscope undertakes to provide a power of attorney to a Divestiture Agent to ensure the Divestiture Business is sold within a further limited timeframe. Healthscope accepts that such a sale is to occur on terms necessary, including as to price, to ensure a sale is effected within that further limited period.
- 1.10 To address the Commission's concern in relation to the relationship between the provision of pathology services to public hospitals and community pathology services in the same geographic regions, Healthscope has agreed that it will tender for the provision of pathology services to public hospitals in the relevant regions on terms approved of by the Independent Manager of the Divestiture Business and will give the purchaser of the Divestiture Business the option to acquire from Healthscope any contract for the provision of pathology services to public hospitals in the relevant regions.
- 1.11 The Commission is satisfied that this Undertaking, the primary purpose of which is to divest an independent, viable business in competition with the merged entity, addresses its competition concerns.

2. Commencement of Undertaking

- 2.1 This Undertaking comes into effect when:
 - (a) it has been executed by Healthscope; and
 - (b) it has been accepted by the Commission,
 (Commencement Date).

3. Sale of the Divestiture Business

- 3.1 Healthscope will, after the Control Date and in accordance with the sale processes set out in this Undertaking, divest, or cause the divestiture of, the Divestiture Business as a viable going concern to an Approved Purchaser within the Divestiture Period.
- 3.2 Healthscope will divest the Divestiture Business on terms which include:
- (a) the assignment or transfer to an Approved Purchaser of all assets that comprise the Divestiture Business, including leases, equipment, intellectual property and rights and obligations under material contracts;
 - (b) the transfer of employment to the Approved Purchaser of all employees necessary for the operation of the Divestiture Business at the time of the sale, and the transfer of any service contract necessary for the operation of the Divestiture Business at the time of sale;
 - (c) the provision by Healthscope of services that the Approved Purchaser requests subject to the services being:
 - (i) provided on a transitional basis;
 - (ii) provided on reasonable arm's length terms; and
 - (iii) approved by the Commission;
 - (d) the transfer, subject to law, of all licences or other regulatory approvals to the Approved Purchaser that are required for the operation of the Divestiture Business, including a requirement for Healthscope to obtain any relevant regulatory or other approval for such transfers;
 - (e) that Healthscope will not do anything to discourage any employees or contractors of the Divestiture Business from taking up employment or contracting with the Approved Purchaser, and that Healthscope will take all reasonable steps to facilitate the transfer of those employees or contractors to the Approved Purchaser;
 - (f) that Healthscope will procure that any Independent Manager appointed pursuant to clause 6 continues to operate the Divestiture Business in the ordinary course of business in competition with Healthscope between the Control Date and the Divestiture Date and will not reduce the scope of any services provided by the Divestiture Business;
 - (g) that Healthscope will apply for third party consents to assignment or transfer of material contracts;
 - (h) that Healthscope will do everything within its power to obtain the consent of third parties to the transfer of assets under this Undertaking; and
 - (i) that Healthscope will cooperate with the Approved Purchaser in any application to obtain an Approved Pathology Authority for the Divestiture Business.
- 3.3 If by the Execution Date the Divestiture Business has not been divested, Healthscope will, on the first Business Day of every month from the Execution Date until the Divestiture

Date, provide a written report to the Commission as to the progress made in the divestiture of the Divestiture Business.

4. Sale to an Approved Purchaser

- 4.1 Healthscope must sell the Divestiture Business to an Approved Purchaser and must not authorise the Divestiture Agent to sell the Divestiture Business to a purchaser other than an Approved Purchaser.
- 4.2 At the election of the Approved Purchaser, Healthscope will, as soon as practicable and at no additional cost, assign or otherwise divest to the Approved Purchaser the benefit of any contract to provide pathology services or any right to provide pathology services at or to a hospital or public health service listed at Item 5 of Schedule 2 pursuant to a tender lodged by Healthscope between the Control Date and the Divestiture Date.
- 4.3 Where Healthscope seeks to have a purchaser of the Divestiture Business approved by the Commission, Healthscope must without delay provide the Commission with a Proposed Purchaser Notice in respect of any Proposed Purchaser which Notice provides the Commission with sufficient information to form a view as to the suitability of the Proposed Purchaser including:
 - (a) the name, address, telephone number and any other available contact detail of the Proposed Purchaser(s);
 - (b) a copy of the proposed Sale and Purchase Agreement;
 - (c) a description of the business carried on by the Proposed Purchaser(s) and, with precision, the locations in which that business is carried on;
 - (d) details as to the Proposed Purchaser's experience in the relevant markets; and
 - (e) the names of the owners and directors of the Proposed Purchaser(s).
- 4.4 The Commission may provide Healthscope with a notice (**Approval Notice**) that the Proposed Purchaser is an Approved Purchaser. An Approval Notice must be in writing and must be provided within ten Business Days after receipt of a Proposed Purchaser Notice, or such further period as is required by the Commission and notified to Healthscope. In making its decision to provide an Approval Notice, the Commission will have regard to whether:
 - (a) the Proposed Purchaser is independent of Healthscope and Symbion;
 - (b) the Proposed Purchaser is of good financial standing and has an intention to maintain the Divestiture Business;
 - (c) the Proposed Purchaser is able to conduct the Divestiture Business having regard to, without limitation, regulatory requirements; and
 - (d) the sale of the Divestiture Business to the Proposed Purchaser will address any competition concern held by the Commission.
- 4.5 Healthscope will not challenge the Commission's refusal to issue an Approval Notice.

5. Preservation of Divestiture Business

- 5.1 From the Control Date, Healthscope will not, except in accordance with this Undertaking, sell or transfer any of its interest in, or assets of, the Divestiture Business or remove or make any material adverse change to the structure, attributes, extent or operations of the Divestiture Business.
- 5.2 Between the Control Date and the Divestiture Date, Healthscope will not submit a tender or proposal for the provision of pathology services pursuant to a contract, arrangement or understanding, to any of the hospitals or public health services listed in Schedule 2 without the consent of the Independent Manager.
- 5.3 Healthscope will from the Control Date until the Divestiture Date, take all steps to ensure that:
- (a) subject to the best interests of the Divestiture Business (which is to be determined by the Independent Manager), it is not involved in the management and operation of the Divestiture Business;
 - (b) the Independent Manager is able to manage and operate the Divestiture Business as a separate, fully operational and competitive going concern and in such a way that preserves the value (including goodwill) of the Divestiture Business; and
 - (c) to the maximum extent practicable, the Divestiture Business is financially and operationally separate from Healthscope.
- 5.4 Without limiting the operation of clause 5.3, Healthscope will, from the Control Date until the Divestiture Date, take all steps to ensure that:
- (a) any Approved Pathology Authority issued by the Minister for Health and Ageing pursuant to the *Health Insurance Act 1973* (Cth) in respect of the Divestiture Business continues in force and is not suspended, withdrawn, revoked or allowed to lapse;
 - (b) any approvals granted under the *Health Insurance (Eligible Collection Centres) Approval Principles* and the *Health Insurance Act 1973* (Cth) in relation to the Approved Collection Centres that constitute part of the Divestiture Business continue in force and are not suspended, withdrawn, revoked or allowed to lapse;
 - (c) any approvals granted under the *Health Insurance (Accredited Pathology Laboratories - Approval) Principles* and the *Health Insurance Act 1973* (Cth) in relation to the Accredited Pathology Laboratories that constitute part of the Divestiture Business continue in force and are not suspended, withdrawn, revoked or allowed to lapse;
 - (d) the Accredited Pathology Laboratories that constitute part of the Divestiture Business continue to be operated and supervised in a manner consistent with the requirements for a laboratory of that category as set out in Part 4 of the *Health Insurance (Accredited Pathology Laboratories - Approval) Principles*; and

- (e) all approvals granted by the National Association of Testing Authorities Australia in relation to the Accredited Pathology Laboratories that constitute part of the Divestiture Business continue in force and are not suspended, withdrawn, revoked or allowed to lapse.
- 5.5 Healthscope will, within five Business Days of the Control Date, notify the Commission of any changes to the Divestiture Business that Healthscope intends to make to ensure the separation of the Divestiture Business required by clause 5.3. Healthscope will within five Business Days implement any additional changes notified to Healthscope by the Commission as being necessary in order to comply with clause 5.3.
- 5.6 In complying with the obligations in this clause 5 Healthscope will, from the Control Date:
- (a) continue to provide access to reasonable working capital and sources of credit for the Divestiture Business;
 - (b) provide and maintain reasonable levels of administrative, promotional, technical, advertising and marketing support to the Divestiture Business;
 - (c) ensure that the Independent Manager is able to acquire and pay for the sufficient and timely delivery of all goods and services (including from third parties) which the Independent Manager considers are required by the Divestiture Business;
 - (d) accept (and direct its directors, officers, employees, contractors and agents to accept) direction from the Independent Manager as to the control, management, financing and operations of the Divestiture Business, and for the Divestiture Business to meet all legal, corporate, financial, accounting, taxation, audit and regulatory obligations;
 - (e) provide and pay for any external expertise, assistance or advice required by the Independent Manager to perform their role;
 - (f) ensure that the Divestiture Business has, at Healthscope's cost, access to and use of the personnel required by the Divestiture Business to operate as a viable going concern;
 - (g) subject to specific approval by the Independent Manager, ensure that, between the Control Date and the Divestiture Date, any Healthscope personnel (including contractors) concerned with the management or operation of the Divestiture Business are not concerned with the management or operation of any aspect of Healthscope's business other than the Divestiture Business;
 - (h) not procure, promote or encourage the redeployment of personnel working at the Divestiture Business as at the Control Date to any other business operated by Healthscope;
 - (i) take any steps directed by the Commission in relation to matters arising from the report of any Independent Manager, within ten Business Days of being so directed;
 - (j) not, at any time from the Control Date and within one year of the Divestiture Date, use any confidential information gained through the ownership and/or management of the Divestiture Business; and

- (k) direct its personnel including directors, managers, officers, employees and agents to act in accordance with the prohibitions and duties in this clause 5.6.

5.7 In respect of Healthscope's Wangaratta Business, references to the Control Date in this clause 5 are to be read as references to the Commencement Date.

6. Appointment of Independent Manager

6.1 Healthscope must appoint an Approved Independent Manager who will manage the Divestiture Business as Independent Manager in accordance with this Undertaking and will maintain that appointment until the Divestiture Date.

6.2 The Independent Manager must be a person who is independent of Healthscope and Symbion and will not be independent if, without limitation, he or she is a:

- (a) current employee or officer of Healthscope or Symbion;
- (b) person who has been an employee or officer of Healthscope or Symbion in the past 18 months;
- (c) shareholder who, in the opinion of the Commission, holds a material number of shares of Healthscope or Symbion;
- (d) professional adviser of Healthscope or Symbion, whether current or in the past 18 months;
- (e) person who has a contractual relationship with Healthscope or Symbion;
- (f) person who is a supplier of Healthscope or Symbion;
- (g) person who is a material customer of Healthscope or Symbion.

6.3 Healthscope will, within ten Business Days of the Commencement Date, advise the Commission in writing of the identity of the Proposed Independent Manager together with such information that the Commission requires to assess whether the Commission is prepared to consent to the Proposed Independent Manager.

6.4 If, within five Business Days of receipt by the Commission of the information from Healthscope referred to in clause 6.3 above or such further period as is required by the Commission and notified to Healthscope:

- (a) the Commission does not object to the Proposed Independent Manager, the Proposed Independent Manager will be an Approved Independent Manager and Healthscope will appoint the Approved Independent Manager as soon as practicable thereafter on terms approved by the Commission and consistent with the performance by the Independent Manager of its functions under this Undertaking and forward to the Commission a copy of the executed terms of appointment of the Approved Independent Manager; or
- (b) the Commission does object to the Proposed Independent Manager, Healthscope must appoint a person identified by the Commission at its absolute discretion as the Approved Independent Manager on terms approved by the Commission and

consistent with the performance by the Independent Manager of its functions under this Undertaking.

- 6.5 In the event that the Independent Manager resigns or is otherwise unable to continue as the Independent Manager, Healthscope will, within five Business Days or such further period as is required by the Commission and notified to Healthscope, appoint a new Independent Manager who is either approved of in writing by the Commission, or identified by the Commission, and whose appointment will be maintained by Healthscope until the Divestiture Date on terms approved by the Commission and consistent with the performance by the Independent Manager of its functions under this Undertaking.
- 6.6 Healthscope will procure that:
- (a) the Independent Manager will operate the Divestiture Business in the ordinary course of business and must not make any material change to the structure, attributes, extent or operations of the Divestiture Business;
 - (b) to the maximum extent practicable, the Independent Manager will operate the Divestiture Business in a manner which is financially and operationally separate from Healthscope;
 - (c) the Independent Manager will keep the books and records of the Divestiture Business separate from those of Healthscope;
 - (d) the Independent Manager will implement specific measures to maintain the confidentiality of any competitively sensitive information of the Divestiture Business;
 - (e) the Independent Manager will use their best endeavours to renew or replace upon expiry material contracts for the provision of goods or services to the Divestiture Business on commercial terms favourable to the Divestiture Business;
 - (f) the Independent Manager will maintain appropriate personnel levels and ensure that the Divestiture Business has access to sufficient personnel to operate as a viable going concern and may engage personnel (including professional advisers) as the Independent Manager determines to be necessary;
 - (g) the Independent Manager will not use any confidential information gained through the ownership and/or management of the Divestiture Business other than for performing their functions as Independent Manager;
 - (h) the Independent Manager will provide a written report each month to the Commission in relation to the operation of the Divestiture Business and this Undertaking over the preceding month, and the Independent Manager will carry out the Commission's directions in relation to matters arising from the report;
 - (i) the Independent Manager must review and report to the Commission regarding any changes made to the Divestiture Business by Healthscope to ensure separation of the Divestiture Business required by clause 5.3, and make any recommendations to the Commission that the Independent Manager considers appropriate;

- (j) the Independent Manager will only give their consent pursuant to clause 5.2 if the Independent Manager forms the view that the terms of the tender or proposal in relation to which consent is being sought is on terms that would be favourable to any Approved Purchaser of the Divestiture Business;
 - (k) the Independent Manager will provide information requested by the Commission directly to the Commission; and
 - (l) the Independent Manager may report or otherwise inform the Commission directly of any issues that arise in the performance of their functions as Independent Manager or in relation to any matter that may arise in connection with this Undertaking.
- 6.7 Healthscope will indemnify the Independent Manager for any expenses, loss, claim or damage arising from the performance by the Independent Manager of functions required to be performed by the Independent Manager by this Undertaking.
- 6.8 The Commission may approve any proposal by, and alternatively may direct, Healthscope to terminate the Independent Manager if the Independent Manager acts inconsistently with the provisions of this Undertaking or the terms of their engagement. On termination, Healthscope will, within five Business Days of the Commission nominating an alternative Independent Manager, appoint the Independent Manager on terms approved by the Commission and consistent with the performance by the Independent Manager of its functions under this Undertaking.
- 6.9 Healthscope will provide to the Independent Manager any information requested by the Independent Manager that the Independent Manager considers necessary for the running of the Divestiture Business or for reporting to or otherwise advising the Commission.
- 6.10 Healthscope will not request any information relating to the Divestiture Business from the Independent Manager without such a request having been made through the Commission.

7. Failure to divest within the Divestiture Period

- 7.1 If the sale of the Divestiture Business to an Approved Purchaser is not completed by the end of the Divestiture Period, the Divestiture Business is an Unsold Business and the provisions of this clause 7 apply.
- 7.2 Within five Business Days of the Divestiture Business becoming an Unsold Business, Healthscope will notify the Commission in writing of the identity of a Proposed Divestiture Agent who is independent of Healthscope and has appropriate experience and/or qualifications to act as a Divestiture Agent, together with such information as the Commission requires regarding the appointment of the Proposed Divestiture Agent, including the proposed terms of appointment.
- 7.3 The terms upon which the Divestiture Agent is appointed must be approved by the Commission and must include terms to the effect that the Divestiture Agent:

- (a) is empowered and required by Healthscope to effect the divestiture of the Unsold Business to an Approved Purchaser within three months of the Appointment Date (or such further period as determined by the Commission at its absolute discretion) at the best price that is attainable within that period subject to an overriding and unconditional obligation to sell the Unsold Business at no minimum price by the end of this period;
- (b) may charge such fees as are agreed between the Divestiture Agent and Healthscope (but not contingent on the price to be obtained for the Unsold Business), to be paid by Healthscope. If agreement cannot be reached between the Divestiture Agent and Healthscope within five Business Days from the date of the Commission's notice that it does not object to the Proposed Divestiture Agent, Healthscope will pay such fees as are reasonably directed by the Commission;
- (c) is the only person who may effect the divestiture of the Unsold Business after the Appointment Date;
- (d) may at Healthscope's expense retain any lawyer, agent or other adviser required to effect the divestiture of the Unsold Business;
- (e) will account to Healthscope for:
 - (i) any moneys derived from the divestiture of the Unsold Business;
 - (ii) all disbursements, fees and charges incurred by the Divestiture Agent in undertaking his/her duties; and
 - (iii) all fees of the Divestiture Agent (including the fees of any advisor appointed under clause 7.3(d));
- (f) will provide a written report on the first Business Day of each month until the Divestiture Date to the Commission and Healthscope, or answer any reasonable enquiries of either the Commission or Healthscope, concerning:
 - (i) the efforts made to effect the divestiture of the Unsold Business;
 - (ii) the identity of any advisers engaged;
 - (iii) any costs incurred;
 - (iv) the identity of any persons expressing interest in the Unsold Business; or
 - (v) any other information reasonably required by the Commission or Healthscope; and
- (g) will use best endeavours to ensure that Healthscope complies with its obligations as set out in this Undertaking and notify the Commission if it becomes aware of any material failure by Healthscope to do so.

7.4 If within ten Business Days of receipt by the Commission of the information referred to in clause 7.2 (or such further period as is required by the Commission and notified to Healthscope in writing prior to the expiration of the ten Business Day period) the Commission objects to the Proposed Divestiture Agent, then Healthscope will appoint an alternative Divestiture Agent nominated by the Commission within ten Business Days of such nomination.

- 7.5 If the Commission does not object to the Proposed Divestiture Agent within ten Business Days of receipt by the Commission of the information referred to in clause 7.2 (or such further period as is required by the Commission and notified to Healthscope in writing prior to the expiration of the ten Business Day period), then Healthscope will, within five Business Days of that date, appoint the Proposed Divestiture Agent as Divestiture Agent.
- 7.6 Healthscope will grant the Divestiture Agent an irrevocable power of attorney conferring all necessary power and authority to effect the divestiture of the Unsold Business on terms which are considered by the Divestiture Agent to be consistent with this Undertaking.
- 7.7 Healthscope must not authorise the Divestiture Agent to sell the Unsold Business to a purchaser other than an Approved Purchaser.
- 7.8 Healthscope must not contract to sell the Unsold Business on terms which would be inconsistent with the Divestiture Agent's role, the granting of authority to the Divestiture Agent under clause 7.6, or any other obligation in this Undertaking.
- 7.9 Healthscope will provide a copy of the executed terms of appointment for the Divestiture Agent to the Commission within five Business Days of their execution.
- 7.10 Healthscope must provide the Divestiture Agent with all relevant information available to Healthscope and reasonably assist the Divestiture Agent to effect the divestiture of the Unsold Business as quickly as possible.
- 7.11 The Commission may request information from the Divestiture Agent directly at any time. Healthscope undertakes to require the Divestiture Agent to report and respond to the Commission directly in response to any such request, or as otherwise required by the Commission.
- 7.12 The Commission must approve any proposal by, and alternatively may direct, Healthscope to terminate the appointment of the Divestiture Agent if the Divestiture Agent acts inconsistently with the provisions of this Undertaking. On termination of the Divestiture Agent, Healthscope must appoint an alternative Divestiture Agent nominated by the Commission within ten Business Days of such nomination.

8. Release of personnel

- 8.1 Where:
 - (a) the Divestiture Business or Unsold Business is divested as contemplated by this Undertaking; and
 - (b) the terms of the divestment contemplate that, immediately following the completion of the sale, Healthscope personnel including staff or persons under a service agreement will be employed by, or otherwise provide services to, the Divestiture Business or Unsold Business,

Healthscope must release the relevant personnel, with effect from the Divestiture Date, from:

- (c) any obligation to provide services to Healthscope; and
- (d) any non-compete or similar restraint of trade obligation, to the extent that such obligation would otherwise prevent the staff member or person from performing his or her contemplated role in relation to the Divestiture Business or Unsold Business.

8.2 Healthscope will not directly or indirectly discourage personnel including staff of, or persons under service agreements with, the Divestiture Business or Unsold Business from continuing in or seeking employment with, or providing services to, the Approved Purchaser of the Divestiture Business or Unsold Business.

9. Provision of information to the Commission

9.1 Healthscope will advise the Commission in writing of the date of the Control Date within one Business Day of the Control Date.

9.2 Healthscope will provide the Commission with copies of any executed Sale and Purchase Agreement within five Business Days of it being entered into.

9.3 At the Commission's direction, Healthscope will, and will procure that its directors, employees, agents or contractors identified by the Commission will:

- (a) furnish information to the Commission;
- (b) produce documents to the Commission within Healthscope's custody, control or power; and/or
- (c) attend the Commission at a time and place appointed by the Commission to answer any questions the Commission, its Commissioner, staff or agents may have;

as directed by the Commission in relation to this Undertaking.

9.4 Nothing in 9.3 requires the provision of information in respect of which Healthscope has a claim of legal professional privilege.

10. Obligation to procure

10.1 Where performance of an obligation under this undertaking requires a Related Body Corporate of Healthscope to take or refrain from taking some action, Healthscope will use its best endeavours to procure that Related Body Corporate to take or refrain from taking that action as the case may be.

11. Confidentiality and disclosure

11.1 Subject to clause 11.2, Healthscope acknowledges that the Commission will make this Undertaking available for public inspection and that the Commission will from time to time publicly refer to this Undertaking.

11.2 Healthscope and the Commission agree that Confidential Schedule 1 to this Undertaking will remain confidential until the Divestiture Date.

11.3 Nothing in clause 11.2 prevents the Commission from disclosing such information as:

- (a) is required by law;
- (b) is reasonably necessary for the purpose of enforcement action under section 87B of the Act; or
- (c) is necessary for the purpose of making such market inquiries as the Commission thinks fit to assess the impact on competition arising in connection with the divestment of the Divestiture Business or the Unsold Business.

12. Costs

All costs incurred by Healthscope in providing information to the Commission, in responding to Commission inquiries as required by this Undertaking and in complying with this Undertaking are to be paid by Healthscope.

13. Notices

13.1 Any notice, demand, consent or other communication given or made under this Undertaking to the Commission should be sent to:

Australian Competition & Consumer Commission
Attention: General Manager, Mergers and Asset Sales
Address: 23 Marcus Clarke Street, Canberra ACT 2601
Fax No: (02) 6243 1212

or to any other address which the Commission subsequently notifies to Healthscope.

11.2 Any notice, demand, consent or other communication given or made under this Undertaking to Healthscope should be sent to the address or fax number below or the address or fax number last notified by the intended recipient to the sender:

Healthscope Limited
Attention: Chief Financial Officer
Address: Level 1, 407 Royal Parade, Parkville VIC 3052
Fax No: (03) 9356 7599

or to any other address which Healthscope subsequently notifies to the Commission.

14. Definitions and Interpretation

Definitions

14.1 The meanings of the terms used in this document are as follows:

Act means the *Trade Practices Act 1974* (Cth).

Accredited Pathology Laboratory	means a laboratory that has been accredited as an Accredited Pathology Laboratory by the Minister for Health and Ageing pursuant to the <i>Health Insurance Act 1973</i> and the <i>Health Insurance (Accredited Pathology Laboratories – Approval) Principles</i> as amended from time to time.
Albury Business	means Symbion's pathology business conducted at Albury, New South Wales, as set out in item 4 of Schedule 2.
Appointment Date	means the date on which a Divestiture Agent is appointed under clause 7.
Approval Notice	means a notice given by the Commission pursuant to clause 4.4.
Approved Collection Centre	means an eligible collection centre approved by the Minister for Health and Ageing pursuant to the <i>Health Insurance Act 1973</i> and the <i>Health Insurance (Eligible Collection Centres) Approval Principles</i> as amended from time to time.
Approved Independent Manager	means a person approved pursuant to clause 6 by the Commission to act as Independent Manager.
Approved Purchaser	means a Proposed Purchaser in relation to which the Commission issues an Approval Notice pursuant to clause 4.4.
Approved Pathology Authority	means a person approved by the Minister for Health and Ageing as an Approved Pathology Authority pursuant to the <i>Health Insurance Act 1973</i> .
Benalla Business	means Symbion's pathology business conducted at Benalla, Victoria, as set out in item 2 of Schedule 2.
Business Day	means a day on which all banks are open generally in Melbourne, but does not include a Saturday, Sunday or public holiday.
Commencement Date	means the date determined in accordance with clause 2.1.
Commission	means the Australian Competition and Consumer Commission.
Control Date	means the date on which the Scheme is implemented or Healthscope otherwise gains control of Symbion.
Divestiture Agent	means a person appointed under clause 7 to effect the divestiture of the Unsold Business.
Divestiture Business	means the businesses identified in Schedule 2.
Divestiture Date	means the date on which Healthscope no longer has ownership or control of the Divestiture Business.
Divestiture Period	means the period described in Confidential Schedule 1 to this Undertaking.
Execution Date	means the date one Business Day after the Control Date.
Gippsland Business	means Symbion's pathology business conducted at Gippsland,

	Victoria, as set out in item 1 of Schedule 2.
Healthscope	means Healthscope Limited (ACN 004 405 152). Unless the context otherwise indicates, a reference in this Undertaking to Healthscope includes a reference to any of its related bodies corporate.
Healthscope Investments	means Healthscope Investments Pty Ltd (ACN 125 694 251).
Independent Manager	means the Approved Independent Manager appointed by Healthscope in accordance with clause 6 to be the Independent Manager of the Divestiture Business.
Proposed Acquisition	means the proposed acquisition of Symbion by Healthscope pursuant to the Scheme.
Proposed Divestiture Agent	means a person proposed by Healthscope to be the Divestiture Agent.
Proposed Independent Manager	means a person proposed by Healthscope to the Commission pursuant to clause 6 to act as the Independent Manager.
Proposed Purchaser	means a proposed purchaser of the Divestiture Business in respect of whom Healthscope has provided the Commission with a Proposed Purchaser Notice.
Proposed Purchaser Notice	means the notice referred to in clause 4.3.
Related body corporate	has the meaning given to that phrase by section 4A of the Act.
Sale and Purchase Agreement	means an agreement or agreements in respect of the sale and purchase of the Divestiture Business.
Scheme	has the meaning given to it in clause 1.3.
Symbion	means Symbion Health Limited (ACN 004 073 410). Unless the context otherwise indicates, a reference in this Undertaking to Symbion includes a reference to any of its related bodies corporate.
Unsold Business	has the meaning given to that phrase in clause 7.1.
Undertaking	is a reference to all of the provisions of this document, including its Schedules.
Wangaratta Business	means Healthscope's pathology business conducted at Wangaratta, Victoria, as set out in item 3 of Schedule 2.

Interpretation

14.2 The following rules of interpretation apply:

- (a) the singular includes the plural, and vice versa;
- (b) a gender includes all genders;

- (c) referring to anything after the words include, includes or including does not limit what else might be included;
 - (d) a reference to "this Undertaking" is a reference to all the provisions of this document;
 - (e) a reference to dollars and \$ is to Australian currency; and
 - (f) all references to time are to time in Melbourne, Australia.
- 14.3 In the interpretation of a clause of this Undertaking, a construction that would promote the purpose or object underlying this Undertaking (whether that purpose or object is expressly stated in this Undertaking or not) shall be preferred to a construction that would not promote the purpose or object.
- 14.4 In the interpretation of this Undertaking, material not forming part of this Undertaking may be considered if that material is capable of assisting in the ascertainment of the meaning or context of a clause of this Undertaking in order to:
- (a) confirm that the meaning of a clause is the ordinary meaning conveyed by the text of the clause taking into account its context in this Undertaking and the competition concerns intended to be addressed by this Undertaking and the clause in question; or
 - (b) determine the meaning of a clause when the ordinary meaning conveyed by the text of the clause taking into account its context in this Undertaking and the purpose or object underlying this Undertaking leads to a result that does not promote the purpose or object underlying this Undertaking.
- 14.5 In determining whether consideration should be given to any material in accordance with clause 13.4, or in considering any weight to be given to any such material, regard shall be had, in addition to any other relevant matters, to:
- (a) the effect that reliance on the ordinary meaning conveyed by the text of the clause would have (taking into account its context in this Undertaking and whether that meaning promotes the purpose or object of this Undertaking); and
 - (b) the need to ensure that the result of this Undertaking is to address the Commission's competition concerns.
- 14.6 The Commission may authorise the Mergers Review Committee, a member of the Commission or a member of the Commission staff, to exercise a decision making function under this Undertaking on its behalf and that authorisation may be subject to any conditions which the Commission may impose.
- 14.7 This Undertaking does not prevent the Commission from taking enforcement action at any time during or after the period of this Undertaking in respect of any breach by Healthscope of any term of this Undertaking.
- 14.8 Nothing in this Undertaking restricts the right of the Commission to take action under the Act for penalties, divestiture or any other remedy/s in the event that divestiture of the Divestiture Business is not implemented in accordance with this Undertaking.

Executed by Healthscope Limited ACN 006
405 152 in accordance with section 127 of the
Corporations Act by or in the presence of:



Signature of Secretary/Director

INGRID PLAYER

Name of Secretary/Director in full

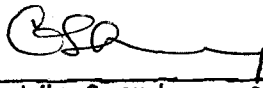


Signature of Director

DAVID R. DIXON

Name of Director in full

Accepted by the Australian Competition and Consumer Commission pursuant to Section 87B
of the Trade Practices Act 1974.



Graeme Julian Samuel
Chairman

15 August 2007

Date

Confidential Schedule 1 – Divestiture Period

Schedule 2 – Divestiture Business

The Divestiture Business comprises:

1. The Gippsland pathology business conducted by Symbion from its Accredited Pathology Laboratories at the following locations:

Sale	Macalister Street
Warragul	Landsborough Street
Leongatha	Koonwarra Road
Bairnsdale	Day Street
Wonthaggi	Graham Street
Traralgon	Princes Highway

and including 27 Approved Collection Centres at the following locations:

Bairnsdale	Day Street
Lakes Entrance	649 Esplanade
Orbost	Boundary Road
Paynesville	6 Fleischer Street
Leongatha	Koonwarra Road
Leongatha	32 Jeffrey Street
Mirboo North	4-6 Brennan Street
Maffra	168 Johnson Street
Sale	Palmerston Street
Sale	284 Raymond Street
Churchill	13 Georgina Place
Moe	16 Kirk Street
Morwell	241 Princes Drive
Neerim South	Main Road
Pakenham	56 Main Street
Traralgon	Princes Highway
Traralgon	6 Kay Street
Yarram	121 Commercial Road
Drouin	22-26 Princes Way
Warragul	40 Albert Street
Cowes	12 Wardley Avenue
Foster	95 Station Street
Inverloch	11 Williams Street
Kooweerup	Rossiter Road
Korumburra	50 Radovick Street
San Remo	123 Marine Parade
Wonthaggi	Graham Street

2. The Benalla pathology business conducted by Symbion from its Accredited Pathology Laboratory at Coster Street, Benalla, including the Approved Collection Centres at the following locations:

Benalla	Coster Street
Mansfield	49 Highett Street
Mooroopna	69 McLennan Street

3. The Wangaratta pathology business conducted by Healthscope from its Accredited Pathology Laboratory at Part Clark Street, Wangaratta, including the Approved Collection Centres at the following locations:

Wangaratta	61 Ovens Street
Yarrawonga	25 Piper Street

4. The Albury pathology business conducted by Symbion from its Accredited Pathology Laboratory at 1/2 Ramsay Place, Albury, including the Approved Collection Centres at the following locations:

Albury	1/2 Ramsay Place
Albury	552 Englehardt Street
Beechworth	39 Camp Street
Corryong	Kiell Street
Corowa	30 Evelyn Street
Lavington	3/334-336 Griffith Road
Mount Beauty	Tawonga Crescent
Wodonga	101 Hume Street
Wodonga	Nordsvan Drive

5. At the discretion of the Approved Purchaser, any contract, or right to enter a contract, for the provision of pathology services at any of the following hospitals or to any of the following public health services secured between the Control Date and the Divestiture Date:

Benalla and District Memorial Hospital
 Heyfield Bush Nursing Hospital
 Latrobe Regional Hospital
 Maffra District Hospital
 Mansfield District Hospital and North Eastern Pathology
 Maryvale Private Hospital
 South Gippsland Hospital
 Wangaratta District Base Hospital
 Wonthaggi and District Hospital
 Bairnsdale Regional Health Service

Gippsland Southern, Central Gippsland and West Gippsland Health Services

Omeo District Health Service

Orbost Regional Health Service

Yarram and District Health Service

Wodonga Regional Health Service

Alpine Health Service